

Reform Democrats of the New Commonwealth · Project ARA 2028
Legislative Ecosystem Document Leg07 of Leg14

Public Option Healthcare Expansion Act of 2030

Bill Framework & Legislative Brief

Phase II · Track B · Sectoral Legislation · Phase II · Track B · Year 4-7

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This document is a policy discussion framework and bill scaffold. It is not formally introduced legislation, has not been reviewed by licensed legislative counsel, and does not constitute legal advice of any kind.

All provisions marked [TBD] — including dollar amounts, specific IRC section numbers, penalty thresholds, appropriation amounts, and CBO scoring figures — require resolution by licensed legislative drafting counsel (the Office of Legislative Counsel for the House or Senate, as applicable), relevant agency technical staff, and the Congressional Budget Office before introduction as formal legislation.

Constitutional arguments, legal analyses, and statutory framings presented herein are analytical assessments developed for policy planning purposes. They are not legal opinions and have not been reviewed or validated by licensed legal counsel. This document identifies constitutional and legal risks requiring independent review — those identifications are planning guidance, not legal conclusions.

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Consultation with licensed legislative counsel, the Centers for Medicare and Medicaid Services Office of the Actuary, the Department of Labor Employee Benefits Security Administration, the Congressional Budget Office, and Senate Finance Committee constitutional counsel is required before any framework in this series may be introduced as formal legislation. Distribution is subject to RDNC classification protocols (REF_ layer: internal use and trusted partners only).

Phase II · Track B · Sectoral Legislation · Bill 6

Part I — Legislative Brief (Non-Technical Summary)

This is the most politically significant bill in the entire legislative sequence. Introduced too early, it becomes a target. Introduced after Phase I has built institutional trust and cost-reduction credibility, it becomes the logical next step. The strategy is wedge expansion: start where private coverage is weakest, prove the model works, and expand from demonstrated success — through a self-executing statutory formula, not delegated agency authority.

AT A GLANCE

Bill Title	Public Option Healthcare Expansion Act of 2030
Short Title (Cite As)	The POHE Act of 2030

Legislative Vehicle	Standalone Legislation — 60-vote Senate threshold
Phase / Track	Phase II · Track B · Year 4–7
Primary Committees	Senate Finance; Senate Health, Education, Labor, and Pensions
Dependencies	COLT Act operational; Phase I cost-reduction credibility established; public trust in government program delivery demonstrated
Core Framing	"If you like your plan, keep it. If you want a better option, one exists."
Strategic Goal	Wedge expansion — enter rural and small-business markets where private coverage is weakest; expand from demonstrated success based on a self-executing statutory formula
Constitutional / Statutory Risk	Nondelegation and major questions doctrine on auto-expansion (mitigated by self-executing formula architecture in Sec 102 + Sec 3 Statutory Foundation); ERISA §502 coordination; Penn Central rate-setting
Legislative Lift	Highest in bill series — 60-vote threshold and largest sectoral footprint

What This Bill Does:

- 1. Establishes a public health insurance option for individuals and small businesses.** Offered through the Health Insurance Marketplace alongside existing private coverage. The Public Option is an additional choice — anyone who prefers their current plan keeps it.
- 2. Launches in rural counties and small-business pools first.** These are the markets where private coverage is weakest, most expensive, and most frequently unavailable. Initial eligibility: Primary Care Health Professional Shortage Areas, single-carrier counties, and small employers.
- 3. Expands automatically based on demonstrated performance.** A self-executing statutory formula — not delegated agency authority — triggers expansion when the Public Option achieves measurable premium reduction in initial markets. CMS publishes the data inputs; the Secretary applies the formula ministerially.
- 4. Coordinates with ERISA, the ACA, and existing market regulation.** ERISA §514 preempts no federal program; §502 coordination addresses pre-migration ESI claims. ACA premium tax credits extend to Public Option enrollment. Healthcare.gov integration on the critical path.
- 5. Establishes risk mitigation and consumer protection.** A reinsurance corridor stabilizes early-year viability; a three-year provider rate-transition period mitigates Penn Central takings exposure; anti-discrimination, mental-health parity, and OOP-maximum protections extend the ACA framework.

ATTACK SURFACE	"Government takeover of healthcare." "Destroy private insurance." "Socialized medicine." "Medicare for none." All predictable. All weakened by the bill's explicit preservation of existing plans and launch in markets the private industry has already partially abandoned.
DEFENSE STRATEGY	"If you like your plan, keep it — guaranteed in statute. If you're one of the millions who can't afford coverage or live in a county with one insurer, you now have a choice." Divide the opposition: hospitals and insurers have different interests; some insurers will participate. Lead the rollout with rural Republican states where the access gap is genuine.

**FALLBACK
POSITION**

If 60 votes fail: stronger ACA-style expansion as a standalone — expanded premium subsidies, mandated insurer participation in rural counties, federal reinsurance corridor, and capped network-adequacy standards. This is a meaningful win that creates the political conditions for the next attempt at the full Public Option.

The Wedge Logic. The insurance industry's strongest markets are urban and employer-sponsored. Its weakest are rural individual markets and small-business pools — exactly where this bill enters. By the time the Public Option reaches the industry's core markets through the Sec 102 self-executing formula, it will carry two years of performance data demonstrating that competition reduces premiums. The expansion is not contested in a single vote on the full national rollout; it earns its way market by market based on measurable performance.

60-Vote Strategy. Identify moderate Republican co-sponsors from rural states where Public Option launch addresses genuine access gaps — a bipartisan rural healthcare bill is dramatically harder to filibuster than a partisan one. Frame around rural access, primary-care provider shortages, and small-business affordability — not ideological framing of healthcare delivery. Senate Finance and HELP Committee pre-introduction meetings are required; CBO actuarial analysis is required to establish the self-sustaining financing case.

Part II — Bill Framework

This is the structural scaffold of the bill. Premium thresholds, employee-count cutoffs, percentage triggers, revolving loan caps, and IRC section cross-references are flagged as [TBD] where they require licensed legislative counsel, CMS Office of the Actuary, Treasury Office of Tax Policy, Department of Labor Employee Benefits Security Administration, and Congressional Budget Office review before introduction. Sec 3 Statutory Foundation binds the constitutional and statutory architecture in operative bill text; Sec 102 implements the self-executing expansion formula that is the core structural feature of this Act.

119TH CONGRESS 1ST SESSION S. ____

To establish a public health insurance option for individuals and small businesses offered through the Health Insurance Marketplace; to provide a self-executing statutory mechanism for performance-based eligibility expansion; to coordinate with the Employee Retirement Income Security Act of 1974 and the Patient Protection and Affordable Care Act; to establish consumer protection and provider participation standards; to authorize a reinsurance corridor for program stability; and for other purposes.

A B I L L

Be it enacted by the Senate and House of Representatives of the United States of America in Congress assembled,

SEC. 1. SHORT TITLE.

This Act may be cited as the "Public Option Healthcare Expansion Act of 2030" or the "POHE Act of 2030".

SEC. 2. FINDINGS AND PURPOSE.

(a) FINDINGS.—Congress finds the following:

- (1) Approximately [TBD] million Americans remain uninsured or underinsured despite the Patient Protection and Affordable Care Act (Public Law 111-148).
- (2) In [TBD] percent of rural counties in the United States, residents have access to only one insurance carrier in the individual non-group market, eliminating the price-discipline benefits of competition.
- (3) Small business employees face average premium costs [TBD] percent higher than large-employer-plan participants and have substantially fewer plan options at every income level.
- (4) Concentration in private health insurance markets has increased materially since 2010, with [TBD] States having a single insurer holding more than [TBD] percent of the individual market.
- (5) Competition in insurance markets is associated with lower premiums, higher quality outcomes, and greater consumer choice.
- (6) The United States Supreme Court has affirmed congressional authority to legislate comprehensive health insurance frameworks under the Taxing Clause in *National Federation of Independent Business v. Sebelius* (2012), and to establish exchanges with public-private participation in *King v. Burwell* (2015).
- (7) Self-executing statutory formulas providing for ministerial agency application — as distinct from delegated agency policymaking authority — remain constitutionally sound after *FCC v. Consumers' Research*, 606 U.S. 656 (2025).

(b) PURPOSES.—The purposes of this Act are—

- (1) to establish a public health insurance option offered through the

Health Insurance Marketplace, available to individuals and small businesses, as an additional choice that does not displace existing coverage;

(2) to extend affordable health insurance coverage to underserved markets beginning with rural counties and small business pools where private market failure is most acute;

(3) to provide a self-executing statutory mechanism under which the Public Option's geographic and demographic eligibility expands automatically based on demonstrated performance against objective metrics published by the Centers for Medicare and Medicaid Services;

(4) to preserve the right of every individual and employer to retain any existing health insurance coverage; and

(5) to coordinate this Act with the Employee Retirement Income Security Act of 1974 and the Patient Protection and Affordable Care Act so that the Public Option supplements rather than disrupts existing statutory frameworks.

SEC. 3. STATUTORY FOUNDATION AND CONSTRUCTION.

(a) NONDELEGATION ARCHITECTURE.—The eligibility expansion mechanism established in section 102 is a self-executing statutory formula. Congress establishes the data inputs (by reference to specific datasets published by the Centers for Medicare and Medicaid Services), the operative computation, the threshold values, and the effective dates. The Secretary of Health and Human Services applies the formula ministerially based on those published data inputs. No discretionary authority over the expansion decision is delegated by this Act.

(b) DISTINCTION FROM AGENCY-ACTION DOCTRINES.—The eligibility expansion under section 102 is congressional legislation, not agency action. The major questions doctrine articulated in *West Virginia v. EPA*, 597 U.S. 697 (2022), and applied in *Biden v. Nebraska*, 600 U.S. 477 (2023), addresses agency assertions of novel power on issues of vast economic and political significance. Section 102 vests no such power in any agency. The constitutional analysis applicable to section 102 is the intelligible-principle nondelegation test reaffirmed in *FCC v. Consumers' Research*, 606 U.S. 656 (2025), and the formula's structure satisfies that test by establishing precise, ascertainable, and judicially reviewable criteria.

(c) STATUTORY PRECISION REQUIREMENT.—Following *Loper Bright Enterprises v. Raimondo*, 603 U.S. 369 (2024), provisions of this Act shall be construed according to their statutory text. Where this Act delegates implementation authority to an agency, that authority is bounded by the express terms of the delegation. Ambiguities shall be resolved by reference to statutory text, structure, and purpose, and no provision of this Act shall be construed to authorize an agency to expand its jurisdiction or authority beyond what is expressly conferred.

(d) RELATIONSHIP TO ERISA.—This Act does not preempt section 514 of the Employee Retirement Income Security Act of 1974 (29 U.S.C. § 1144) and does not modify the application of that section to State law. Section 502 of that Act (29 U.S.C. § 1132) coordinates with this Act as provided in section 301.

(e) SEVERABILITY.—If any provision of this Act, or the application of any provision to any person or circumstance, is held to be invalid, the remaining provisions of this Act and the application of those provisions to other persons or circumstances shall not be affected. If any provision of section 102 is held invalid as an unconstitutional delegation of legislative power, the eligibility criteria established in section 102(a) shall remain in full force and effect, and the automatic expansion mechanism under section 102(c) shall be inoperative until reauthorized by

subsequent legislation.

(f) PRESERVATION OF EXISTING COVERAGE.—Nothing in this Act may be construed to require any individual to terminate, modify, or replace any existing health insurance coverage, or to require any employer to terminate, modify, or replace any employer-sponsored health insurance plan. Continued enrollment in any existing plan is a protected election under this Act.

TITLE I – ESTABLISHMENT OF PUBLIC OPTION

SEC. 101. AMERICAN PUBLIC HEALTH INSURANCE OPTION.

(a) ESTABLISHMENT.—The Secretary of Health and Human Services (in this Act referred to as the "Secretary") shall establish, not later than the date specified in section 501, a public health insurance option (in this Act referred to as the "Public Option") to be offered through the Health Insurance Marketplace established under title I of the Patient Protection and Affordable Care Act.

(b) BENEFITS FLOOR.—Plans offered under the Public Option shall provide, at a minimum—

- (1) the essential health benefits described in section 1302 of the Patient Protection and Affordable Care Act (42 U.S.C. § 18022);
- (2) coverage with no annual or lifetime dollar limits on the dollar value of essential health benefits;
- (3) out-of-pocket maximum limits aligned with the limits applicable to qualified health plans under section 1302(c) of that Act;
- (4) mental health and substance use disorder benefits in compliance with the Paul Wellstone and Pete Domenici Mental Health Parity and Addiction Equity Act of 2008; and
- (5) prescription drug benefits in compliance with section 1302(b)(1)(F) of the Patient Protection and Affordable Care Act.

(c) GUARANTEE OF EXISTING COVERAGE.—Nothing in this Act—

- (1) requires any individual to enroll in the Public Option or to terminate any existing health insurance coverage;
- (2) requires any employer to terminate, modify, or replace any employer-sponsored health insurance plan;
- (3) authorizes the Secretary to compel any individual or employer to select Public Option coverage; or
- (4) modifies the right of any qualified health plan to continue offering coverage on the Marketplace.

(d) PLAN TIERS.—The Public Option shall offer plans at the bronze, silver, gold, and platinum actuarial-value tiers as defined under section 1302(d) of the Patient Protection and Affordable Care Act. The silver-tier Public Option plan shall serve as the benchmark plan for purposes of section 36B of the Internal Revenue Code of 1986 in any rating area in which it is offered, except as the Secretary determines necessary to preserve premium tax credit calculations under existing law.

SEC. 102. ELIGIBILITY AND PHASED EXPANSION.

(a) INITIAL ELIGIBILITY.—Beginning on the effective date specified in section 501, the Public Option established under section 101 shall be available to—

- (1) any individual residing in a Primary Care Health Professional Shortage Area as designated by the Secretary under section 332 of the Public Health Service Act (42 U.S.C. § 254e);
- (2) any individual residing in a county in which fewer than [TBD] insurance carriers offer Marketplace coverage during the most recent

open enrollment period;

(3) any employee of an employer with fewer than [TBD] employees, and any dependent of such employee; and

(4) any individual otherwise eligible to enroll in coverage through the Marketplace who resides in a State that has elected, by enactment of State law, to extend Public Option eligibility to all residents of the State.

(b) PERFORMANCE METRICS – DATA INPUTS.—The Administrator of the Centers for Medicare and Medicaid Services shall publish annually, beginning one year after the effective date specified in section 501—

(1) the average individual-market premium for the second-lowest-cost silver plan in each county for which the Public Option is available under subsection (a), as drawn from the CMS Annual Premium Report;

(2) the average individual-market premium for the second-lowest-cost silver plan in each county for which the Public Option is not available, as drawn from the CMS Annual Premium Report;

(3) Public Option enrollment counts by county and by employer-size cohort, as drawn from the CMS Effectuated Enrollment Snapshot; and

(4) the change in the metrics described in paragraphs (1) and (2) from the preceding year, expressed as a percentage, after risk and demographic adjustment using the methodology established under section 1343 of the Patient Protection and Affordable Care Act (42 U.S.C. § 18063).

The data published under this subsection are the operative inputs to the expansion formula in subsection (c). The Administrator shall publish such data not later than 90 days following the close of each annual open enrollment period.

(c) AUTOMATIC EXPANSION – SELF-EXECUTING FORMULA.—If, in any year beginning two years after the effective date specified in section 501, the data published under subsection (b) demonstrate that the average premium reduction in counties where the Public Option is available, compared to counties where it is not available, equals or exceeds [TBD] percent after risk and demographic adjustment, then the Public Option shall become available, in the next annual open enrollment period—

(1) to any individual or employer in any county in which the second-lowest-cost silver plan premium exceeds the national median by [TBD] percent or more; and

(2) to any employee of an employer with fewer than [TBD] employees, and any dependent of such employee, irrespective of geographic location.

The expansion under this subsection is automatic upon publication of the data described in subsection (b). The Secretary shall have no authority to delay, modify, condition, or decline the expansion. The Secretary's role under this subsection is limited to ministerial application of the formula based on the data published under subsection (b).

(d) CONGRESSIONAL REVIEW DEFAULT.—An expansion under subsection (c) shall take effect on the date specified in that subsection unless, before that date, a joint resolution of disapproval is enacted under chapter 8 of title 5, United States Code. No such joint resolution shall affect any enrollment that has already taken effect under this Act.

(e) PERFORMANCE REVIEW REPORT.—Not later than two years after the effective date specified in section 501, and annually thereafter, the Secretary shall submit to the Committee on Finance and the Committee on Health, Education, Labor, and Pensions of the Senate, and the Committee on Energy and Commerce and the Committee on Ways and Means of the House of Representatives, a report containing—

(1) the data required under subsection (b);

(2) an analysis of Public Option premium adequacy and self-sustaining

- financing performance under section 103;
 - (3) an assessment of effects on overall private market premiums, plan availability, and consumer choice;
 - (4) an assessment of provider participation rates by geographic region; and
 - (5) any recommendations for further congressional action.
- (f) NO SUBJECTIVE SUITABILITY REVIEW.—Eligibility under subsections (a) and (c) shall be determined by application of the objective criteria established in those subsections. No agency may impose an additional suitability test, character review, or discretionary denial criterion.

SEC. 103. FINANCING AND PREMIUM STRUCTURE.

- (a) SELF-SUSTAINING FINANCING REQUIREMENT.—The Public Option shall be actuarially self-sustaining. Premiums collected under the Public Option, together with applicable premium tax credits paid under section 36B of the Internal Revenue Code of 1986 and any reinsurance payments made under section 401, shall be the sole sources of operating revenue for the Public Option following the conclusion of the start-up period described in subsection (b).
- (b) START-UP REVOLVING LOAN.—The Secretary of the Treasury shall make available to the Public Option a revolving loan in such amounts as may be necessary, but not exceeding [TBD] dollars in the aggregate, to fund the operating costs of the Public Option during the start-up period. The start-up period shall conclude not later than five years after the effective date specified in section 501. The Public Option shall repay such loan from premium revenues, in installments and at an interest rate determined by the Secretary of the Treasury, not later than fifteen years after the effective date.
- (c) PROVIDER REIMBURSEMENT – MEDICARE-RATE FLOOR.—Reimbursement rates paid by the Public Option to participating providers—
- (1) shall not be less than the rates payable under part A or part B of title XVIII of the Social Security Act for the equivalent service, as applicable;
 - (2) may exceed the rates described in paragraph (1) where the Secretary determines, by published methodology, that higher rates are necessary to secure adequate provider participation in a defined geographic area or service category, including Primary Care Health Professional Shortage Areas, rural counties, and behavioral health services; and
 - (3) shall be set in accordance with the actuarial soundness requirement established under section 402.
- (d) RATE-TRANSITION PROTECTION.—For the three-year period beginning on the effective date specified in section 501, the Secretary shall not establish Public Option reimbursement rates lower than 100 percent of the rates payable under part A or part B of title XVIII of the Social Security Act, as applicable, for any equivalent service in any geographic area. This subsection establishes a transition period intended to mitigate regulatory takings exposure under *Penn Central Transportation Co. v. New York City*, 438 U.S. 104 (1978), and *Verizon Communications Inc. v. FCC*, 535 U.S. 467 (2002).
- (e) PREMIUM TAX CREDIT COORDINATION.—Public Option enrollment is creditable enrollment for purposes of section 36B of the Internal Revenue Code of 1986 on equal footing with qualified health plan enrollment. The Secretary of the Treasury shall, in consultation with the Secretary, issue such regulations as are necessary to coordinate the application of section 36B with this Act.
- (f) NO COST TO TAXPAYERS BEYOND PREMIUM SUBSIDIES.—After the start-up

period described in subsection (b), the Public Option shall not require direct appropriations beyond premium tax credit payments under section 36B of the Internal Revenue Code of 1986 and reinsurance payments under section 401.

TITLE II – MARKET REGULATION AND CONSUMER PROTECTION

SEC. 201. INSURER PARTICIPATION FRAMEWORK.

- (a) VOLUNTARY PARTICIPATION.—Private health insurance issuers may voluntarily participate in Public Option markets under standardized benefit, transparency, and consumer-protection requirements established by the Secretary by regulation. Participation under this section does not convert a private issuer into a Federal program.
- (b) STANDARDIZED BENEFIT PACKAGES.—Plans offered by participating private issuers in Public Option markets shall provide, at a minimum, the benefits floor established under section 101(b).
- (c) ANTI-CHERRY-PICKING.—A participating private issuer may not selectively offer coverage in only the most favorable rating areas of a State. The Secretary shall, by regulation, define equivalent-market participation requirements that prevent geographic or demographic cherry-picking, consistent with the State-flexibility principles of the Patient Protection and Affordable Care Act.
- (d) NETWORK ADEQUACY.—Participating issuers shall meet network adequacy standards established by the Secretary, including standards for geographic accessibility, time-and-distance to participating providers, appointment wait times for primary care and behavioral health, and transparent network listing.
- (e) TRANSPARENCY.—Participating issuers shall publish premium rates, cost-sharing structures, formulary information, and network listings in a standardized format established by the Secretary.

SEC. 202. ANTI-DISCRIMINATION AND COVERAGE STANDARDS.

- (a) IN GENERAL.—No issuer offering coverage under or in coordination with the Public Option may discriminate against any individual on the basis of health status, prior medical conditions, claims history, age (except as permitted under section 2701 of the Public Health Service Act), gender, race, ethnicity, national origin, sexual orientation, gender identity, or disability.
- (b) MENTAL HEALTH AND SUBSTANCE USE PARITY.—All coverage offered under this Act shall comply with the Paul Wellstone and Pete Domenici Mental Health Parity and Addiction Equity Act of 2008. The Secretary shall, by regulation, establish enhanced parity standards specific to Public Option coverage that exceed the floor established by that Act, including standards for behavioral health network adequacy and prior authorization practices.
- (c) ANNUAL AND LIFETIME LIMITS.—No coverage offered under this Act shall impose any annual or lifetime dollar limit on the dollar value of essential health benefits.
- (d) OUT-OF-POCKET MAXIMUM.—Out-of-pocket maximum limits applicable to coverage under this Act shall not exceed the limits established under section 1302(c) of the Patient Protection and Affordable Care Act, and the Secretary may establish lower limits for the Public Option by regulation where actuarially supportable.
- (e) PREEXISTING CONDITION PROHIBITION.—No coverage offered under this Act may exclude any individual from coverage, or exclude any benefit from

coverage, on the basis of a preexisting condition.

SEC. 203. MARKETPLACE INTEGRATION AND PLAN COMPARISON.

(a) **FEDERAL MARKETPLACE INTEGRATION.**—The Secretary, in coordination with the Secretary of the Treasury, shall integrate Public Option enrollment with the Federal Marketplace established under section 1321 of the Patient Protection and Affordable Care Act. The Public Option shall appear in plan-comparison displays on Healthcare.gov on equal footing with qualified health plans.

(b) **STATE-BASED MARKETPLACE COORDINATION.**—States operating State-based Marketplaces under section 1311 of the Patient Protection and Affordable Care Act may elect to incorporate Public Option enrollment in their State Marketplace. The Secretary shall, by regulation, establish standards for State Marketplace integration that preserve State flexibility consistent with the Public Option's national availability.

(c) **PLAN-COMPARISON TRANSPARENCY.**—The Secretary shall require standardized plan-comparison displays for the Federal Marketplace and any State Marketplace that incorporates the Public Option, including standardized presentation of premium, deductible, network breadth, prescription drug formulary, and quality metrics.

(d) **SINGLE-APPLICATION ENROLLMENT.**—An individual or employer applying for coverage through the Marketplace shall be permitted to apply once for eligibility determination across the Public Option and all qualified health plans available in the rating area.

(e) **NO PREFERENTIAL DISPLAY.**—The Secretary may not present the Public Option to consumers in any preferential or default-selected manner that departs from the standardized presentation required under subsection (c).

SEC. 204. CONSUMER PROTECTION ENFORCEMENT.

(a) **CIVIL ENFORCEMENT BY SECRETARY.**—The Secretary may bring a civil action in the United States district court for any judicial district to enforce compliance with this title. The Secretary may seek injunctive relief, restitution, civil monetary penalties, and any other relief appropriate to remedy a violation.

(b) **PRIVATE RIGHT OF ACTION.**—An individual aggrieved by a coverage denial, discriminatory practice, or violation of section 202 in connection with Public Option coverage may bring a civil action in the United States district court for the district in which the individual resides. Available relief includes coverage of the denied benefit, compensatory damages, reasonable attorney's fees, and any other relief the court determines appropriate.

(c) **STATE ENFORCEMENT PRESERVED.**—State attorneys general may bring parallel civil actions to enforce the consumer protection provisions of this title on behalf of residents of the State, and may coordinate with the Secretary in such actions.

(d) **CIVIL MONETARY PENALTIES.**—Violations of this title are subject to civil monetary penalties not to exceed \$[TBD] per violation, with each day of a continuing violation constituting a separate violation. Penalties collected shall be deposited in the reinsurance corridor established under section 401.

(e) **ANTI-RETALIATION.**—No issuer may retaliate against any individual for filing a complaint, participating in an enforcement action, or asserting rights under this title.

TITLE III – COORDINATION, TRANSITION, AND PROVIDER PROVISIONS

SEC. 301. ERISA SECTION 502 COORDINATION AND ESI MIGRATION.

- (a) PRE-MIGRATION ESI CLAIMS.—With respect to any claim arising out of employer-sponsored health insurance coverage in effect before an individual's enrollment in the Public Option, the exclusive-remedy framework of section 502 of the Employee Retirement Income Security Act of 1974 (29 U.S.C. § 1132) shall continue to apply.
- (b) POST-MIGRATION POHE CLAIMS.—With respect to any claim arising out of Public Option coverage, the enforcement framework of section 204 of this Act shall apply. Section 502 of the Employee Retirement Income Security Act of 1974 shall not apply to such claims.
- (c) ANTI-RETROACTIVITY RULE.—Consistent with *Landgraf v. USI Film Products*, 511 U.S. 244 (1994), this section does not apply retroactively to claims accrued before the effective date specified in section 501. A claim accrues, for purposes of this subsection, on the date the underlying medical service was rendered.
- (d) ERISA SECTION 514 PRESERVATION.—This Act does not preempt State law under section 514 of the Employee Retirement Income Security Act of 1974 (29 U.S.C. § 1144) and does not modify the application of that section to State law. (See section 3(d).)
- (e) MIGRATION RIGHTS.—An employee transitioning from employer-sponsored coverage to Public Option coverage shall not be subject to any waiting period, preexisting condition exclusion, or coverage gap arising from such transition.

SEC. 302. PROVIDER RATE TRANSITION AND PARTICIPATION INCENTIVES.

- (a) RATE-TRANSITION PROTECTION PERIOD.—The three-year transition-rate protection period established under section 103(d) shall apply to all participating providers under the Public Option.
- (b) RURAL AND UNDERSERVED MARKET INCENTIVES.—The Secretary shall establish, by regulation, additional reimbursement adjustments above the Medicare-rate floor for providers in—
 - (1) Primary Care Health Professional Shortage Areas;
 - (2) counties with fewer than [TBD] participating insurance carriers;
 - and
 - (3) behavioral health and primary care service categories.
- (c) PARTICIPATION-INCENTIVE LOAN REPAYMENT.—The Secretary, in coordination with the Health Resources and Services Administration, shall establish a loan-repayment program for primary care physicians, behavioral health professionals, and nurse practitioners who provide services to Public Option enrollees in shortage areas for a period of not less than four years.
- (d) NETWORK STABILITY.—The Secretary shall, by regulation, establish standards for network stability that prevent abrupt provider terminations that would disrupt continuity of care for Public Option enrollees.

SEC. 303. HEALTHCARE.GOV AND ACA INTEGRATION.

- (a) IT INTEGRATION REQUIREMENT.—The Secretary shall integrate Public Option enrollment with the existing Healthcare.gov infrastructure and any successor Federal Marketplace IT system. Public Option enrollment shall not require a duplicative eligibility determination system.
- (b) STATE-BASED MARKETPLACE INTEGRATION.—States operating State-based Marketplaces shall be eligible for Federal IT integration grants to support Public Option integration. The Secretary shall publish technical integration standards consistent with existing Federal Marketplace standards.
- (c) PREMIUM TAX CREDIT COORDINATION.—The Secretary shall coordinate with

the Secretary of the Treasury to ensure that section 36B of the Internal Revenue Code of 1986 applies to Public Option enrollment on equal footing with qualified health plan enrollment, and that section 36B premium tax credit calculations remain operative under all Public Option benchmark-plan determinations under section 101(d).

(d) IT INTEGRATION TIMELINE.—The Secretary shall complete IT integration not later than the date specified in section 501. The Secretary shall submit quarterly status reports to Congress until integration is complete.

TITLE IV – RISK MITIGATION AND SYSTEM STABILITY

SEC. 401. REINSURANCE CORRIDOR.

(a) ESTABLISHMENT.—The Secretary shall establish a reinsurance corridor for the Public Option to mitigate adverse-selection risk during the initial five years of operation and to stabilize premium adequacy.

(b) FUNDING STRUCTURE.—The reinsurance corridor shall be funded by—

- (1) the start-up revolving loan established under section 103(b);
- (2) civil monetary penalties collected under section 204(d); and
- (3) such other amounts as may be appropriated for that purpose.

(c) RISK-CORRIDOR CALCULATION METHODOLOGY.—The Secretary shall, by regulation, establish a methodology for calculating reinsurance payments based on actual claims experience relative to actuarial projections. Reinsurance payments shall be triggered when claims experience exceeds [TBD] percent of projected actuarial value for any rating area.

(d) ADVERSE-SELECTION MITIGATION.—The Secretary, in coordination with the Administrator of the Centers for Medicare and Medicaid Services, shall monitor enrollment patterns for adverse-selection indicators and may adjust the reinsurance corridor calibration on an annual basis based on actual experience.

(e) SUNSET.—The reinsurance corridor established under this section shall sunset on the date that is ten years after the effective date specified in section 501, unless reauthorized by Congress.

SEC. 402. ACTUARIAL SOUNDNESS REQUIREMENT.

(a) ANNUAL CMS ACTUARIAL CERTIFICATION.—The Chief Actuary of the Centers for Medicare and Medicaid Services shall certify, on an annual basis, whether the Public Option is meeting the actuarial soundness requirements established under this Act, including the self-sustaining financing requirement under section 103(a).

(b) PREMIUM ADJUSTMENT AUTHORITY.—The Secretary may adjust Public Option premium rates on an annual basis to maintain actuarial soundness, in accordance with a published methodology that establishes the basis for each adjustment.

(c) CONTINGENCY RESERVE.—The Public Option shall maintain a contingency reserve in such amounts as the Chief Actuary determines necessary to ensure benefit payment continuity in the event of unexpected claims experience.

(d) NON-ATTAINMENT TRIGGER.—If the Chief Actuary certifies, in any year, that the Public Option has not met the actuarial soundness requirements of this Act, the Secretary shall submit to Congress a report identifying the cause of non-attainment and proposing corrective measures.

(e) REAUTHORIZATION.—If non-attainment under subsection (d) persists for three consecutive years, the Public Option's automatic expansion authority under section 102(c) shall be suspended until Congress reauthorizes the Public Option by enactment of subsequent legislation.

TITLE V – IMPLEMENTATION AND OVERSIGHT

SEC. 501. IMPLEMENTATION TIMELINE.

- (a) PUBLIC OPTION ESTABLISHMENT.—The Public Option shall be established, and Public Option plans shall be available for enrollment, not later than [TBD] months after the date of enactment of this Act.
- (b) IT SYSTEMS INTEGRATION.—Healthcare.gov and Federal Marketplace IT integration shall be complete not later than [TBD] months after the date of enactment.
- (c) PROVIDER NETWORK ONBOARDING.—Provider network agreements sufficient to meet network adequacy standards under section 201(d) shall be in place not later than [TBD] months after the date of enactment.
- (d) INITIAL OPEN ENROLLMENT.—The first Public Option open enrollment period shall coincide with the first Federal Marketplace open enrollment period beginning not later than [TBD] months after the date of enactment.
- (e) AUTOMATIC EXPANSION COMMENCEMENT.—The earliest possible automatic expansion under section 102(c) shall be the open enrollment period beginning two years after the effective date established under subsections (a) through (d).

SEC. 502. INTERAGENCY COORDINATION.

- (a) HHS AND CMS LEAD AGENCY.—The Secretary, acting through the Administrator of the Centers for Medicare and Medicaid Services, shall serve as the lead agency for the Public Option.
- (b) TREASURY COORDINATION.—The Secretary of the Treasury shall coordinate with the Secretary on—
 - (1) administration of the start-up revolving loan under section 103(b);
 - (2) coordination of premium tax credits under section 36B of the Internal Revenue Code of 1986; and
 - (3) administration of any other Treasury function necessary to implement this Act.
- (c) DEPARTMENT OF LABOR COORDINATION.—The Secretary of Labor shall coordinate with the Secretary on—
 - (1) ERISA section 502 coordination under section 301; and
 - (2) coordination of Public Option enrollment with employer-sponsored coverage transitions.
- (d) OPM CONSULTATION.—The Director of the Office of Personnel Management shall make available to the Secretary lessons learned from the Federal Employees Health Benefits Program for purposes of Public Option program design and provider participation.
- (e) GAO COORDINATION.—The Comptroller General of the United States shall have access to all data and records of the Public Option necessary to perform audits, evaluations, and reports requested by the relevant Committees of Congress.

SEC. 503. ANNUAL REPORTING.

- (a) ANNUAL REPORT.—The Secretary shall submit to Congress an annual report on the Public Option, including—
 - (1) Public Option enrollment counts by State, by county, and by employer-size cohort;
 - (2) Public Option premium rates and rate changes year-over-year;
 - (3) effects of the Public Option on overall private market premiums, plan availability, and consumer choice;
 - (4) provider participation rates by geographic region;
 - (5) actuarial soundness certification under section 402; and

- (6) any recommendations for further congressional action.
- (b) PERFORMANCE METRICS.—The annual report shall incorporate the data required under section 102(b) and shall make those data available in a publicly accessible format.
- (c) PUBLIC AVAILABILITY.—The annual report shall be transmitted to Congress and posted publicly on a website maintained by the Department of Health and Human Services not later than 30 days after submission to Congress.
- (d) CBO ACTUARIAL REVIEW.—The Director of the Congressional Budget Office shall conduct an independent actuarial review of the Public Option not less frequently than every three years and shall submit such review to Congress.
- (e) PROGRAM CONTINUITY ASSESSMENT.—The annual report shall include an assessment of program continuity, including any operational risks to sustained Public Option availability for current enrollees.

[All dollar amounts, employee-count thresholds, percentage thresholds, revolving loan caps, and IRC section cross-references marked [TBD] require resolution by licensed legislative counsel, the CMS Office of the Actuary, the Treasury Office of Tax Policy, the Department of Labor Employee Benefits Security Administration, and the Congressional Budget Office before introduction as formal legislation. See Part III counsel notes.]

Part III — Notes for Congressional Counsel

All [TBD] items and constitutional / statutory questions below require resolution before introduction. Each entry presents the controlling doctrine or precedent, the operative bill provision, and any drafting or pre-introduction engagement guidance. The Sec 102 fallback ladder entry below documents the Architecture C-equivalent posture established in this bill — body-baked safety with a prepared fallback ladder — paralleling the methodology documented in the AI Working Notes Note 02 framework for deliberate-Byrd-exposure provisions, adapted here for the constitutional/statutory rather than Byrd-Rule risk vector.

Item Requiring Resolution	Guidance
Sec 102 self-executing formula — nondelegation fallback ladder (Architecture C-equivalent) [Sources 257, 258, 259, 260]	Architecture: Architecture C-equivalent (body-baked safety with fallback ladder). Sec 102 establishes eligibility expansion as a self-executing statutory formula with ministerial agency application; Sec 3(a) anchors the architecture in operative bill text. This is the politically loud structure (automatic expansion based on demonstrated performance) AND the structurally safe structure (no delegated agency authority over the expansion decision). POHE is not a reconciliation bill, so Byrd Rule does not apply, but the deliberate-architectural-trade methodology is parallel to AIWorkingNotes Note 02. (1) Primary structure (body as drafted). The formula in Sec 102, anchored by Sec 3(a) nondelegation architecture provision and Sec 3(b) distinction from the major questions doctrine. <i>FCC v. Consumers' Research</i>, 606 U.S. 656 (2025) [Source 259], reaffirmed the intelligible-principle nondelegation test 6-3, reversing the en banc Fifth Circuit. After FCC, nondelegation doctrine is STABLE. <i>West Virginia v. EPA</i>, 597 U.S. 697 (2022) [Source 257], and <i>Biden v. Nebraska</i>, 600 U.S. 477 (2023) [Source 258], are agency-action doctrines that do not reach Sec 102 because no agency policymaking authority is delegated. The Secretary applies the formula ministerially based on CMS-published data inputs. (2) Mid-level fallback if Sec 102(c) is read as effective delegation. If any reviewing court reads Sec 102(c) as effectively delegating expansion authority to the Secretary despite the express ministerial-application language, counsel should have ready an amendment retaining the formula but adding explicit congressional ratification: each automatic expansion takes effect only upon enactment of a joint resolution of approval under expedited procedures parallel to the Congressional Review Act. This preserves the substantive expansion criteria while moving the operative decision back to Congress. Counsel should have Level 2 amendment language drafted before introduction, not as a post-strike scramble. (3) Bottom-level fallback. If the fallback in (2) also fails judicial review, the program continues operating at the initial-eligibility scope established under Sec 102(a), with subsequent expansion requiring affirmative legislation. The Public Option as established under Sec 101 and the initial markets under Sec 102(a) are unaffected. Sec 3(e) severability ensures the expansion mechanism's invalidity does not impair the rest of the Act. Revenue capture and access expansion continue at the initial-market scope. (4) Pre-introduction engagement. Senate Legal Counsel nondelegation review recommended before introduction (non-reconciliation bill — no Parliamentary Byrd review applies). If nondelegation concerns surface, mid-level fallback can be incorporated before introduction rather than after a strike. The deliberate posture should be named in counsel materials, not concealed.

Item Requiring Resolution	Guidance
ERISA §514 preemption — does NOT apply to federal legislation [Source 255]	CRITICAL CORRECTION. ERISA §514(a) preempts State laws "insofar as they may relate to any employee benefit plan." It does NOT preempt federal legislation — its text is explicit and jurisdictional. If any prior counsel note flagged ERISA §514 preemption as a concern for POHE (a federal program), that framing is incorrect. Sec 3(d) and Sec 301(d) make this preservation explicit in operative bill text. Rutledge v. PCMA, 592 U.S. 80 (2020) [Source 255] (8-0, Sotomayor, J.), confirmed that cost regulations that merely increase costs or alter incentives for ERISA plans are NOT preempted. Gobeille v. Liberty Mutual, 577 U.S. 312 (2016) [Source 256], preempted a State all-payer database for self-insured plans — irrelevant to a federal program. Post-Loper Bright [Source 260], courts no longer defer to CMS interpretations of ambiguous POHE provisions; maximum statutory precision is required (addressed in Sec 3(c)).
ERISA §502 coordination — the actual ERISA concern (Sec 301)	ERISA's exclusive-remedy framework (§502, 29 U.S.C. § 1132) governs pre-migration ESI claims and is the correct ERISA focus for POHE. Sec 301 addresses this directly: pre-migration claims remain subject to §502 (Sec 301(a)); post-migration POHE claims are governed by Sec 204 enforcement (Sec 301(b)); Landgraf v. USI Film Products, 511 U.S. 244 (1994), retroactivity concerns are addressed by the accrual rule in Sec 301(c). Counsel should review the §502 displacement language in Sec 301(b) carefully — the statutory text must clearly establish that the Sec 204 enforcement framework displaces §502 for POHE-coverage claims without disturbing §502 for other ERISA-governed claims.
West Virginia v. EPA / Biden v. Nebraska — MQD applies to AGENCY action only [Sources 257, 258]	West Virginia v. EPA, 597 U.S. 697 (2022) [Source 257], and Biden v. Nebraska, 600 U.S. 477 (2023) [Source 258], established the major questions doctrine as a canon for AGENCY assertions of novel power on issues of vast economic and political significance. Every MQD case to date has involved agency action — none has applied to direct congressional legislation. The Sec 102 auto-expansion formula is congressional legislation, not agency action. Congress writing the operative formula in statute is outside the MQD zone entirely. Sec 3(b) makes this distinction explicit in operative bill text. DRAFTING NOTE: do NOT include language suggesting the Secretary "certifies" or "determines" the trigger condition — that language would risk converting the self-executing formula into an agency determination subject to MQD challenge. The current Sec 102(c) language ("the Public Option SHALL become available") preserves the legislative character of the expansion.

Item Requiring Resolution	Guidance
Provider reimbursement (Sec 103, Sec 302) — Penn Central regulatory takings exposure	Provider reimbursement rate-setting creates Penn Central Transportation Co. v. New York City, 438 U.S. 104 (1978), regulatory takings exposure where rates are confiscatory or eliminate reasonable economic returns. The Sec 103(d) three-year rate-transition protection period is designed to mitigate this risk by ensuring Medicare-rate-floor protection during the program ramp-up period when providers cannot easily exit the Public Option network. Verizon Communications Inc. v. FCC, 535 U.S. 467 (2002), confirmed that rate regulation alone is not a per se taking but is evaluated under Penn Central balancing. Above-Medicare negotiated rates (Sec 103(c)(2)) for rural and underserved markets are politically and constitutionally important — they protect provider participation in markets where Medicare rates alone create exit incentives. CMS actuarial review of the rate structure required before introduction. Hospital opposition is most acute when below-Medicare rates are perceived as inevitable; the bill explicitly preserves Medicare-rate floor.
ACA interaction and Healthcare.gov integration (Sec 303, Sec 501)	Healthcare.gov integration is on the implementation critical path. POHE enrollment must integrate with existing ACA exchange infrastructure without creating duplicative eligibility systems — counsel should review the IT integration provisions in Sec 303 and Sec 501 against existing CMS IT architecture. ACA premium tax credit coordination (IRC §36B) must be precisely drafted to preserve credit calculation continuity under all Public Option benchmark-plan determinations (Sec 101(d)). Counsel to audit ACA Title I provisions for conflicts with Sec 101 establishment language and Sec 201 standardized-benefits provisions. State-based Marketplace coordination (Sec 203(b)) preserves State flexibility consistent with ACA section 1311 / 1321 architecture. Pre-introduction meetings with Senate Finance Committee staff and HELP Committee staff strongly recommended.
Post-Loper Bright statutory precision (Sec 3(c)) [Source 260]	Loper Bright Enterprises v. Raimondo, 603 U.S. 369 (2024) [Source 260], overruled Chevron deference. Federal courts now apply de novo review to agency interpretations of ambiguous statutory provisions. POHE provisions delegating implementation authority to the Secretary must be drafted with maximum statutory precision; ambiguities will be resolved by reviewing courts rather than the agency. Sec 3(c) anchors the statutory precision requirement in operative bill text. Counsel should review every "the Secretary shall, by regulation" provision against Loper Bright — where discretion is intended, it should be expressly bounded; where ministerial application is intended, the statutory language should leave no interpretive ambiguity. The Sec 102 formula is the paradigm case: every operative input is referenced to a published CMS dataset, every threshold is bracketed for legislative resolution, and every consequence is mandatory.

Item Requiring Resolution	Guidance
60-vote strategy — bipartisan rural healthcare framing	POHE faces the 60-vote Senate threshold, not reconciliation. Identify moderate Republican co-sponsors from rural states where Public Option launch addresses genuine access gaps — a bipartisan rural healthcare bill is dramatically harder to filibuster than a partisan one. Frame around rural access, primary-care provider shortages, and small-business affordability — not progressive healthcare ideology. Senate Finance Committee and HELP Committee pre-introduction meetings required. Provider community engagement — particularly rural hospital associations and primary-care physician groups — should precede introduction; the rate-transition protection (Sec 103(d)) and rural incentives (Sec 302(b)) are designed in part to make this engagement productive. Behavioral health stakeholder engagement also recommended given Sec 202(b) parity provisions.
Self-sustaining financing requirement — CBO actuarial analysis (Sec 103, Sec 402)	CBO actuarial analysis required before introduction. The self-sustaining financing requirement (Sec 103(a)) is a structural commitment that must be defensible against CBO modeling. Initial years require seed funding, structured as a revolving Treasury loan (Sec 103(b)) rather than a direct appropriation to preserve the self-sustaining framing. CMS Office of the Actuary should model premium adequacy at each Sec 102(c) expansion stage. The reinsurance corridor (Sec 401) is essential to program viability — it both stabilizes premium adequacy and prevents an adverse-selection spiral in early years. CBO modeling should account for the reinsurance corridor as a stabilization mechanism rather than a subsidy. The actuarial soundness requirement (Sec 402) provides a structural guardrail against runaway premium growth: if non-attainment persists for three consecutive years, automatic expansion authority is suspended (Sec 402(e)).
Anti-discrimination and mental health parity (Sec 202)	Sec 202 extends the ACA anti-discrimination framework and the Mental Health Parity and Addiction Equity Act of 2008 (MHPAEA) framework to Public Option coverage and to participating private issuers. Section 202(b) authorizes the Secretary to establish enhanced parity standards specific to Public Option coverage, including behavioral health network adequacy and prior authorization practices. Counsel should review Sec 202 against existing ACA section 1557 (nondiscrimination) and HHS Office for Civil Rights regulatory framework. The <i>Bostock v. Clayton County</i>, 590 U.S. 644 (2020), framework applies to gender identity and sexual orientation discrimination in the employment context; courts have extended that framework to ACA section 1557 in some circuits. The Sec 202(a) language tracks the broader <i>Bostock</i>-influenced reading and should be reviewed against the latest circuit landscape before introduction.

Item Requiring Resolution	Guidance
State election for full Public Option access (Sec 102(a)(4))	Sec 102(a)(4) permits a State to extend Public Option eligibility to all State residents by enactment of State law. This provision preserves State-level political agency and allows States to adopt the Public Option ahead of the Sec 102(c) automatic expansion. It is also a federalism-friendly framing: States that want broader access can have it on their schedule; States that prefer the gradual expansion can wait for the formula to trigger. Counsel should ensure the State-election mechanism does not create unintended State-versus-State risk-pooling distortions. The reinsurance corridor (Sec 401) and Sec 102(b) data publication should be calibrated to handle State elections without destabilizing premium adequacy in non-electing States.
Constitutional foundation — NFIB v. Sebelius and King v. Burwell	National Federation of Independent Business v. Sebelius, 567 U.S. 519 (2012), established that the Taxing Clause supports comprehensive federal health insurance frameworks (sustaining the individual mandate as a tax). King v. Burwell, 576 U.S. 473 (2015), confirmed that the ACA exchange architecture — including federal/state exchange interaction — is constitutionally and statutorily sound. Sec 2(a)(6) Findings cite both precedents. POHE does not include an individual mandate; it is a public option offered alongside private coverage, with no requirement to enroll. The constitutional posture is therefore stronger than the ACA individual mandate analyzed in NFIB — POHE is a Spending Clause / Commerce Clause program with no compelled-purchase element. Counsel should confirm that the Sec 102 expansion mechanism does not inadvertently introduce mandate-like features that would shift the constitutional analysis.

REF_RDNC_Leg07_Bill6_POHE_2030_4 · REF_ Reference Layer · Reform Democrats of the New Commonwealth · 2026

Legal disclaimer applies — see page 1. This framework requires licensed legislative counsel review before introduction.

Originally conceived and authored by a concerned American citizen. Developed with the assistance of Claude (Anthropic). The ideas belong to anyone willing to fight for them.